

Brain-Based Mental Health Community

EMDR (Eye Movement Desensitization and Reprocessing) Feedback

NAME

DATE

Review the live website before providing feedback:

<https://tcfasd.github.io/Brain-Based-Mental-Health-/handout-therapy-guide.html>

Ages 4+ | Brain-Based Fit: Moderate With Adaptation

What It Asks the Brain to Do:

EMDR asks the brain to reprocess traumatic memories that have gotten "stuck" in the emotional brain (limbic system) without being properly filed away. The therapist uses bilateral stimulation — eye movements, tapping, or sounds — while the person holds a difficult memory in mind. This appears to help the brain move memories from "active threat" storage to "past event" storage. Research shows changes in brain regions responsible for fear, memory, and regulation after EMDR treatment. The child does not need to talk extensively about the trauma.

Caregiver Involvement:

Varies. For children, best practice includes caregiver involvement in preparation, stabilization, and support between sessions. The actual reprocessing is typically between the child and therapist. Some child-specific EMDR protocols include the caregiver more actively.

How to Know If It Might Be a Good Fit:

- Your child has specific traumatic memories that seem to be driving symptoms
- Talk-based trauma processing hasn't worked because your child can't or won't narrate the experience
- Your child can hold a memory in mind while simultaneously tracking movement or tapping (a dual-attention task)
- The therapist has specific training in EMDR with children and neurodevelopmental adaptations

Brain-Based Consideration

EMDR requires working memory — the ability to hold one thing in mind while doing another. For children with brain-based differences, this dual-attention demand may need to be carefully assessed. Adaptations include simpler language, slower pace, more caregiver involvement, and the "storytelling method" for younger developmental ages. EMDR can be effective for children with FASD who carry trauma, but it should not be a first-line standalone approach — it works best as part of a broader plan with a clinician who understands neurodevelopmental adaptations.

1. Is what is written accurate?

☐

Yes

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No

Yes

2. Is there additional information that you think caregivers should know about EMDR (Eye Movement Desensitization and Reprocessing)?



Thank you for your feedback!